

LABORATORY INVESTIGATION REPORT

Patient Name	: Mrs. CHANDRAMATHI GAJANANA HEGDE	Age/Sex	: 75 Year(s)/Female
UHID	: NMSH,547100	Order Date	: 12/07/2025 18:38
Episode	: OP		
Ref. Doctor	: DR. NARAYANA PANJI	Mobile No	: 9900794242
Address	: D 93 50 FEET ROAD D BLOCK GOPALA GOWDA EXTN , SHIMOGA ,Shimoga,Karnataka ,0	Facility	: Nanjappa Multispeciality

Histopathology

Test	Result	Unit	Reference Range
Sample No : 07004289B3	Collection Date : 14/07/25 11:27	Ack Date : 14/07/2025 11:27	Report Date : 16/07/25 17:48

SMALL SPECIMEN BIOPSY

SPECIMEN NO

H - 1404/25

SPECIMEN DETAILS :

CT guided biopsy of hypoechoic mass in abdomen

CLINICAL DETAILS :

Large hypoechoic lobulated mass in abdomen

USG- Large hypoechoic mass in epigastric region

GROSS EXAMINATION :

Received 8 light brown to dark brown linear cores ranging in length from 0.3 to 0.4cm. (AE) Single block

MICROSCOPIC EXAMINATION :

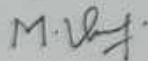
shows multiple cores of tissue with diffuse sheets and nodular aggregates of medium sized lymphoid cells with round hyperchromatic nuclei and scanty cytoplasm. Lymphoid cells are also seen infiltrating into adjacent fat tissue.

IMPRESSION :

Possibility of non hodgkin lymphoma needs consideration.

(IHC correlation is advised)

End of Report



Dr.VENKATESWARALU M

REG.NO. 82590
Consultant Pathologist

DEPARTMENT OF RADIODIAGNOSIS

NAME: MRS. CHANDRAMATHI GAJANANA HEGDE	REFERRED BY: DR. PRITVIRAJ S K
AGE/SEX: 73 Y/F	REGN NO: P126798
DATE:: 12-Jul-2025	RDBNO: 250712110

CECT ABDOMEN AND PELVIS

A preliminary AP topogram of the Abdomen and pelvis was obtained. Volume scans were performed starting from the domes of diaphragm down to the level of pubic symphysis, employing 0.625 mm sections. 80 ml of Optiray 350 mg was used for contrast enhancement.

OBSERVATIONS:

➤ **MESENTERY AND RETROPERITONEUM:**

- There is well defined homogeneously enhancing soft tissue mass noted in the pre and para aortic region of mid abdomen, measuring approximately 112 x 75 x 102mm. No cystic changes / calcification.
 - Superiorly, the lesion is extending upto the origin of superior mesenteric artery and inferiorly upto the bifurcation of aorta.
 - Anteriorly, it is seen displacing the bowel loops.
 - Right laterally, it is seen encasing the inferior vena cava and right renal artery and vein.
 - Left laterally, it is extending and abutting the left renal hilum and encasing the left renal artery and vein.
 - Supero-laterally, it is abutting the tail of pancreas
 - Posteriorly, it is encasing the abdominal aorta, inferior mesenteric artery and origin of superior mesenteric artery. No evidence of vertebral body erosion.
 - Multiple enlarged pre and para-aortic lymph nodes are noted, largest measuring 12 x 10mm.

➤ **LUNG BASES:**

- Similar characteristic homogeneously enhancing soft tissue lesion noted in the posterior mediastinum encasing the left hilum, left pulmonary veins, left main and segmental bronchi and also extending into the right lower lobe of lung, together measuring approximately 126 x 93mm.
- Left mild pleural effusion noted.
- Multiple small nodular lesions scattered in bilateral lung fields, largest measuring 20 x 23mm in posterior segment of right upper lobe.

DEPARTMENT OF RADIO DIAGNOSIS

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CECT ABDOMEN AND PELVIS

➤ **LIVER:**

- Normal in size, shape, outlines and parenchymal attenuation.
- No focal lesion.
- The porta hepatis and portal vein are normal.
- The intrahepatic portal venous radicals are normal.
- No evidence of intrahepatic biliary radicular dilatation.

➤ **GALL BLADDER:**

- Normal in size, shape and outlines. Pericholecystic area is normal. The common bile duct is not dilated.

➤ **SPLEEN:**

- Spleen is normal in size, shape and attenuation values.
- The splenic hilum and splenic vein are normal.

➤ **PANCREAS:**

- Pancreas is normal in size, contour and attenuations values.
- No evidence of focal mass lesion/pancreatic duct dilatation.

➤ **GE JUNCTION:** is normal.

➤ **STOMACH:** shows normal cardia, body & antropyloric region.

➤ **DUODENUM:** is normal. No evidence of wall thickening.

➤ **KIDNEYS:**

- Kidneys are normal in size and shape.
- The renal outlines are normal and both kidneys are showing good concentration and prompt excretion of contrast.
- No evidence of focal mass lesion/hydronephrosis/calculi.
- Adrenal glands are normal in size, shape and attenuation values.
- Aorta and extra hepatic portion of inferior venacava are normal in calibre.

➤ **URINARY BLADDER:** is normal in size and attenuation.

➤ **NO EVIDENCE OF ASCITES.**

➤ **UTERUS AND OVARIES:** Atrophy.

DEPARTMENT OF RADIODIAGNOSIS

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CECT ABDOMEN AND PELVIS

- **BOWEL LOOPS:** Small and large bowel loops are normal.
 - Ileo-caecal junction is normal.
- Degenerative changes noted in thoraco-lumbar vertebral bodies
- Numerous small lytic lesions are noted in the vertebral bodies - Metastasis.

IMPRESSION:

- ❖ Well defined homogeneously enhancing soft tissue mass in the pre and para aortic region (Retroperitoneum) of mid abdomen with extension and encasement as described. No cystic changes / calcification.
 - ❖ Multiple enlarged pre and para-aortic lymph nodes.
 - ❖ Similar characteristic homogeneously enhancing soft tissue lesion in the posterior mediastinum encasing the left hilum, left pulmonary veins, left main and segmental bronchi and also extending into the right lower lobe of lung.
 - ❖ Left mild pleural effusion.
 - ❖ Multiple small nodular lesions scattered in bilateral lung fields.
- DD's
1. Lymphoma.
 2. Metastasis.
- ❖ Numerous small lytic lesions in the vertebral bodies - Metastasis.

→ Suggested Histopathology correlation.


Dr. Kiran Kumar R.G.
MBBS DMRD DNB
Consultant Radiologist
KMC. Reg. No - 87099